Universal Care Guideline

Primary Survey

General Impression
Appearance, Breathing, Activity

Medical: A - B - C
Airway, Breathing, and Circulation

Trauma: C - A – B - C
Catastrophic Hemorrhage/C-Spine, Airway, Breathing, Circulation

Cardiac Arrest: C - A – B
Circulation (Compressions), Airway, Breathing

Manage life threats as they are identified

Consider ALS Back-Up

- If Advanced Life Support Skills are required

Establish Triage Category

Life Threatening
Red Priority

Serious
Yellow Priority

Minor
Green Priority

Baseline Vitals

Vital Signs:
- LOC (AVPU/GCS)
- Respiratory Rate
- Pulse Rate
- Blood Pressure
- Temperature

Cardiac or Respiratory Compromise:
- Pulse oximetry
- 12-lead ECG

Altered Mental Status:
- Blood Glucose

Consider Oxygen

Administer Oxygen
- To maintain an Sp02 of >94%
- And select the appropriate method and rate of oxygen delivery to mitigate or treat hypercarbia associated with hypoventilation.

Secondary Survey

Medical
- Head-to-Toe examination

Trauma
- Rapid Trauma Survey “DCAP BLS TIC”

SAMPLE History

OPQRST Assessment

IV Access

Establish IV Access
- If indicated for interventions, or in patients who are at risk of deterioration.
- **Consider IO access** if IV access cannot be readily obtained

Pain Management

Administer Appropriate Analgesia
- If appropriate

Condition Specific Treatment

Go to Appropriate CPG's
- Provide treatment relevant to the patient's medical condition

Temperature Management

Prevent hypo/hyperthermia
- Especially in severe trauma, sepsis and environmental emergencies.

Patient Reassessment

Reassess Patient
- At least every 5 minutes for critical (red priority patients)
- At least every 15 minutes for non-critical (yellow priority patients)
- At least every 15 minutes for non-critical (green priority patients)

Determine Receiving Facility

Confirm receiving facility with Dispatch 2 prior to departing.
- Consider the appropriate hospital based on the patient condition (e.g., trauma center for severe injuries, stroke center for suspected stroke).
- Consider Airwing for patients with time-critical conditions where ground transport time exceeds 30 minutes.

Patient Handover / Endorsement

Provide a concise patient handover to receiving facility in the IMIST-AMBO format:
- Identification, Mechanism/Medical complaint, Injuries/Illness, Signs, Symptoms, Treatment, Allergies, Medications, Background, Other

Patient Care Goals

1. To ensure safety and stabilize the patient's airway, breathing, and circulation.
2. Provide a standardized, systematic approach to assess and manage all patients in the prehospital setting to ensure timely identification of life-threatening conditions and optimize patient outcomes.
3. Deliver appropriate initial interventions to stabilize patients while facilitating linkage to specific clinical guidelines based on assessment findings.
4. Ensure safe and efficient patient care delivery, prioritizing safety, patient comfort, and effective communication during transfer to definitive care with accurate documentation.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- All patients requiring emergency medical services, who require medical assessment and/or treatment, regardless of complaint or condition.
- Patients with medical, traumatic, or undifferentiated presentations necessitating universal care principles.

Exclusion Criteria

- None noted – Universal care guidelines apply to all patients.

Key Patient Assessment Points

1. **Scene Safety Evaluation and Initial Evaluation**
    - **Safety:** Assess hazards (environmental, violence, chemical exposures) to ensure safety for EMS personnel, patients, and bystanders. Use appropriate personal protective equipment (PPE)
    - **Resource Assessment:** Determine the number of patients and request additional resources if needed. (e.g., ALS backup, mass casualty declaration).
2. **Primary Survey**
    - **General impression:**
        - The general impression is formed from observations on arrival to the patient.
            - Evaluate appearance, breathing, and circulation to form a general impression.
        - In pediatric patients, this is known as the Pediatric Assessment Triangle.
    - **ABC Approach:** Assess Airway, Breathing, and Circulation (ABC) to identify life threatening conditions:
        - Ensure airway patency. (e.g., suction, airway adjuncts).
        - Assess breathing (rate, effort, Sp02)
        - Check pulse, control bleeding, and evaluate consciousness (AVPU or GCS 3-15)

      Adapt ABC Approach as Needed:
        - **Trauma:** Use **C-ABC** (Catastrophic Hemorrhage/C-Spine Control, Airway, Breathing, Circulation)
        - **Cardiac Arrest:** Follow **CAB**: Compressions, Airway, Breathing.
        - **Baseline Vitals:** Record initial vital signs (pulse, BP, respiratory rate, Sp02, temperature, RBS if indicated).
3. **Determine Patient Triage Category**
    - Based upon findings in the primary survey, patient triage may be able to be determined. (e.g. bradycardia, or no breathing) as per Patient Triage Categories CPG.
    - If the triage status is unable to be determined in the primary survey, conduct a secondary survey.
4. **Secondary Survey**
    - Perform a systematic **head-to-toe exam** to identify additional injuries or conditions.
        - **For Trauma:** conduct a **Rapid Trauma Survey** to identify hidden injuries (DCAP BLS TIC – Deformities, Contusions, Abrasions, Punctures/Penetrations, Burns, Lacerations, Swelling, Tenderness, Instability, Crepitus)
        - **For Medical Complaints:** focus on relevant systems (e.g., cardiovascular, neurological)
    - The following aspects should be examined (as appropriate to the medical condition or traumatic injury) in the secondary survey assessment:
        - Head
            - Pupils

        - Ears
        - Naso-oropharynx
        - Skull and scalp
    - Neck
        - Jugular venous distention
        - Tracheal position
        - Spinal Tenderness
    - Chest
        - Retractions
        - Breath sounds
        - Chest wall tenderness, deformity, crepitus, and excursion
        - Respiratory pattern, symmetry of chest movement with respiration
    - Abdomen/Back
        - Tenderness or bruising
        - Abdominal distention, rebound, or guarding
        - Spinal tenderness, crepitus, or step-offs
        - Pelvic stability or tenderness
    - Extremities
        - Pulses
        - Edema
        - Deformity/crepitus
    - Neurologic
        - Mental status/orientation
        - Motor-sensory
    - Evaluate for medical equipment (e.g., pacemaker/defibrillator, left ventricular assist device (LVAD), insulin pump, dialysis fistula)
1. **OPQRST History**
    - Assess symptoms and pain using OPQRST:
        - **O**nset of symptoms
        - **P**rovocation: location: any exacerbating or alleviating factors
        - **Q**uality of pain
        - **S**everity of symptoms: pain scale
        - **T**ime of onset and circumstances around onset
2. **SAMPLE History**
    - Gather history using SAMPLE:
        - **S**ymptoms
        - **A**llergies: medication, environment, and foods
        - **M**edications: prescription and over the counter; bring containers to ED if possible
        - **P**ast medical history:
            - Look for medical alert tags, portable medical records
            - Look for medical devices/implants
            - For females of childbearing age, inquire of potential or recent pregnancy and the date of the first day of her last menstrual cycle
        - **L**ast oral intake
        - **E**vents leading up to the emergency call

5. **Assess Pain**
    - Assess pain as per the Pain Management CPG.
    - Assess pain using a 0-10 pain scale or non-verbal cues.
        - Use Wong-Baker Faces scale for non-verbal patients.
        - Use the FLACC pain scale for Pediatric patients < 3 years.
6. **Mechanism of Injury or Illness Context**

- Identify potential injury mechanisms (e.g., falls, collisions) or sources of illness (e.g., infection, overdose) to guide assessment and treatment.
- Collect pre-arrival information (e.g., dispatch details; bystander reports) to inform clinical decisions.

Patient Management & Interventions

1. **Primary Survey**
   - Conduct a rapid **ABCDE assessment (Airway, Breathing, Circulation, Disability, Exposure)** to identify and address immediate life threats (e.g., airway obstruction, severe hemorrhage)
     - Adapt ABC Approach:
       - **Trauma: C-ABC** (Catastrophic Hemorrhage/C-Spine, Airway, Breathing, Circulation)
       - **Cardiac Arrest: CAB** (compressions, airway, breathing).
     - **Airway:** Open airway (head tilt/chin lift, jaw thrust), suction, and/or insert airway adjuncts (OPA/NPA, supraglottic, ETT)
       - Refer to Airway Management CPG for advanced airways.
       - For patients with laryngectomies or tracheostomies, remove all objects or clothing which may obstruct the opening of these devices. Maintain the flow of prescribed oxygen and reposition the head and/or neck.
     - **Breathing:**
       - Evaluate rate, breath sounds, accessory muscle use, retractions, patient positioning, oxygen saturations.
       - **Provide supplemental oxygen** as appropriate to achieve a Sp02 of 94-98% based on clinical presentation and assessment of ventilation, using appropriate delivery method:
         - Nasal cannula (2-6L/min) for mild hypoxia
         - Non-rebreather mask (10-15L/min) for moderate to severe hypoxia.
         - BVM for inadequate ventilation.
         - Titrate oxygen to avoid hyperoxia, especially in COPD or suspected stroke patients.
       - If the patient is apneic (not breathing) Go to Airway Management CPG
     - **Circulation:**
       - Control any major external bleeding (direct pressure, tourniquets, hemostatic agents)
       - Assess pulse:
         - If no pulse, go to the appropriate Resuscitation CPG.
         - Assess rate and quality of carotid and radial pulses
       - Evaluate perfusion by assessing skin color and temperature
       - Evaluate capillary refill.
     - **Disability:**
       - Initiate manual cervical spine restriction if trauma is suspected, maintaining neutral alignment.
       - Evaluate patient's initial responsiveness using: AVPU
       - Evaluate gross motor and sensory function in all extremities
       - Check blood glucose in patients with altered mental status or suspected stroke. If blood glucose is less than 70mg/dl - go to Hypoglycemia CPG.
       - If acute stroke suspected: Complete BEFAST assessment and go to Stroke CPG
     - **Expose** patient for examination as appropriate to complaint:
       - Be considerate of patient modesty
       - Keep patient warm/cool as required.
2. **Consider Advanced Life Support (ALS) Back-Up**[A]
   - Evaluate the need for ALS back-up:
     - ALS back-up should be considered when advanced skills or interventions are required to treat the patient.
     - ALS back-up should be requested within 5 minutes of arrival at the patient.

- It is appropriate to request for ALS back-up, and later cancel the request if it is not required (e.g. the patient improves, or it is determined that the ambulance will reach hospital significantly faster than the back-up will arrive at the patient).
- When indicated, the request for ALS back-up should be made. Do not assume that no ALS unit is available.
- It is appropriate to request for ALS back-up and rendezvous with the ALS unit en route to hospital.
- **Time to Hospital:**
  - Under normal circumstances, ALS back-up should only be requested if the time for the ALS unit to reach the patient is significantly faster than the time to reach hospital.
  - As a general guideline for most ambulances, where there is a transfer time of less than 10 minutes, and ALS back-up has not already been dispatched: requesting for ALS back-up is usually not appropriate. The patient should be transferred directly to hospital.
    - Extrication time, the ability to adequately perform clinical skills and the potential for the patient to deteriorate should also be factored into time-to-hospital decision making.
- **Unstable Red-Priority Patients:**
  - Generally, all patients categorized as critical or 'red priority' are unstable as they have a high potential to deteriorate and should have ALS back-up requested.
- **Stable Red-Priority Patients:**
  - Some Red Priority patients, while critical, may be categorized as 'stable-red priority' if they:
    - Are being adequately managed with BLS procedures, and/
    - Have acceptable vital signs, and/
    - Are not expected to deteriorate during transfer, and/
    - Do not require any urgent advanced life support interventions.
  - Examples of these cases could be controlled arterial limb bleeding, or an uncomplicated CVA.
  - All other red priority patients should be categorized as 'unstable'. Clinical staff must use sound clinical judgement when determining if a patient may be categorized as stable.
  - It is appropriate to not request for ALS back-up for Stable Red Priority patients.

3. **Obtain a full set of baseline vital signs**
   - This includes: pulse, blood pressure, respiratory rate, temperature, Sp02, Blood glucose measurement (if indicated), EtC02 (if indicated), and neurologic status assessment.
   - **Neurologic status assessment**: establish a baseline and reassess for any change in patient neurologic status
     - Assess AVPU (Alert, responds to Voice, responds only to Pain, Unresponsive)
     - Assess Glasgow Coma Score (GCS).
   - Patients with **cardiac or respiratory compromise**:
     - Pulse oximetry
     - 12-lead electrocardiogram (ECG) should be obtained promptly in patients with cardiac or suspected cardiac complaints.
     - Continuous cardiac monitoring
     - Consider waveform capnography for patients with respiratory complaints (essential for critical patients and those patients who require invasive airway management)
   - Patients with **altered mental status:**
     - Check blood glucose. If <70mg/dl go to Hypoglycemia CPG.
4. **Secondary Survey**
   - If interventions are required in the primary survey, it may not be possible to complete the secondary survey.
   - **Neurologic status assessment**:
     - Establish a baseline and reassess for any change in patient neurologic status.
     - Assess Glasgow Coma Score (GCS).
   - **Physical Examination**
     - Perform a physical examination tailored to the patient's chief complaint or suspected condition, avoiding delays in critical patients.

        - **For Trauma:** conduct a **Rapid Trauma Survey** to identify hidden injuries (DCAP BLS TIC – Deformities, Contusions, Abrasions, Punctures/Penetrations, Burns, Lacerations, Swelling, Tenderness, Instability, Crepitus)
        - **For Medical Complaints:** focus on relevant systems (e.g., cardiovascular, neurological)
    - **Sample History**
        - Gather patient history using SAMPLE (Symptoms, Allergies, Medications, Past history, Last oral intake, Events)
    - **OPQRST Assessment**
        - Assess symptoms and pain using OPQRST (Onset, Provocation, Quality, Radiation, Severity, Time) for pain or symptoms.
5. **Consider Red Flags**
    - Consider findings which may indicate a threat to life or require immediate intervention, such as:
        - Apnea, pulselessness, or severe hemorrhage
        - Altered mental status (AVPU: Voice, Pain, Unresponsive) or neurological deficits (e.g., stroke symptoms, seizures)
        - Hypoxia (SpO2 <94%), hypotension (SBP <90 mmHg), or abnormal vital signs indicating instability.
        - High-risk mechanisms (e.g., fall >10 feet, high-speed crash, electrocution) suggestive of severe injuries.
6. **Vascular Access**
    - Establish IV access if indicated or in patients who:
        - Are at risk for clinical deterioration, or
        - Require IV fluid or medication administration.
    - Intraosseous (IO) access is indicated if IV access fails or is impractical.
    - Monitor for complications (e.g., infiltration, dislodgement) and secure access properly.
7. **Temperature Management**
    - Monitor for hypo/hyperthermia, especially in environmental emergencies, severe trauma and sepsis.
    - Use passive warming or cooling measures as needed.
8. **Pain Management**
    - Administer appropriate analgesia per Pain Management CPG.
9. **Patient Reassessment**[B]
    - Patient Reassessment is required every:
        - Every 5 minutes for critical (red-priority) patients
        - Every 10 minutes for serious (yellow-priority) patients
        - Every 15 minutes for non-critical (green-priority) patients.
    - Reassessment should include:
        - Level of consciousness, airway, breathing, and circulation.
        - Vital signs
            - Routine Blood Glucose reassessment is not required for patients with:
                - Acceptable blood glucose levels, and/
                - No clinical suspicion that hypo or hyperglycemia requires reassessment.
        - Condition specific signs and symptoms
        - Any interventions (e.g. IV line patency)
        - Response to interventions. (e.g. pain level)
        - Monitoring of trends to detect changes in patient condition.
10. **Consider Tele-EMS Consultation**
    - Tele-EMS should be consulted for complex or deteriorating conditions.
11. **Determine Receiving Facility**
    - Consider the appropriate hospital based on the patient condition (e.g., trauma center for severe injuries, stroke center for suspected stroke).
        - Confirm receiving facility with Dispatch 2 prior to departing.

- Consider air medical transport (Airwing), if available, for patients with time-critical conditions where ground transport time exceeds 30 minutes.
    - Airwing will not provide transportation for: imminent birth, or violent patients.

12. Patient Handover/Endorsement

- Provide a concise patient handover to receiving facility in the **IMIST-AMBO** format: (**I**dentification, **M**echanism/**M**edical complaint, **I**njuries/Illness, **S**igns, **S**ymptoms, **T**reatment, **A**llergies, **M**edications, **B**ackground, **O**ther) including:
    - Patient name and age
    - Chief complaint
    - History
    - Medications
    - Allergies
    - Vital Signs
    - Interventions
    - Response to treatment
- Practice clear, structured communication to ensure critical information is relayed accurately to the receiving facility.

Patient Safety Considerations

- Respect the patients' rights:
    - All patients above the age of 18 years old have the right to refuse treatment and transfer to hospital provided they have full mental capacity.
    - Refer to Refusal of Transfer CPG.
- For a patient to have full mental capacity, they must be able to understand, retain and repeat information told to them by you.
- Advanced Care Directives/ Living Will's are not recognized in the prehospital environment in Dubai at the time of publication of this CPG version.
- Evaluate the need for ALS backup or Tele EMS consultation for complex or deteriorating conditions.
- Consider air medical transport, if available, for patients with time-critical conditions where ground transport exceeds 30 minutes.

Documentation

- Complete a comprehensive electronic Patient Care Record (ePCR), capturing the following elements to support continuity of care. Including:
    - Identification: Patient's full name, age, and gender.
    - Mechanism / Medical Complaint: Primary complaint or mechanism of injury/illness prompting EMS activation.
    - Injuries / Information: Relevant assessment findings, red flags, risk factors, vital signs, and initial survey details.
    - Signs: Vital signs, AVPU/GCS, pain score (if relevant), and changes during reassessment.
    - Treatment: All interventions performed, including oxygen therapy, IV/IO access, medications, repositioning, and reassessments.
    - Allergies: Any known allergies identified or confirmed during patient history.
    - Medications: Current prescribed or over-the-counter medications, especially those affecting current condition.
    - Background: Past medical history, recent health events, or factors influencing presentation or care decisions.
    - Other: Scene observations and communication with receiving facility.
- Ensure timestamps for all critical events, assessments, interventions, and reassessments to establish a clear timeline for care.

- Document red flags (e.g., hypoxia, hypotension, altered mental status) and actions taken (e.g., ALS backup requested).
- Ensure that the ePCR contains all details to provide a complete 'picture of events'. Provide additional detail in the additional information section if required.
- Record communication with receiving facility, including IMIST-AMBO handover details and any feedback received.
- Verify ePCR for accuracy, legibility, and completeness before submission, ensuring compliance with DCAS documentation standards.

Key Performance Indicators

1. Apply a torniquet for uncontrolled arterial limb bleeding.
2. Request for ALS back-up made within 1 minute of patient contact (when indicated).

Clinical Insights & Additional Guidance

- **Pediatrics**
  - Pediatrics in Dubai are considered less than or equal to 13 years of age.
  - Use length-based tape (Broselow Tape) to estimate patient weight and guide medication therapy and adjunct choice.
- **Geriatrics**
  - Geriatrics in Dubai are considered greater than or equal to 65 years of age.
  - In geriatric patients, as well as all adult patients, reduced medication dosages may apply to patients with renal disease (i.e., on dialysis or a diagnosis of chronic renal insufficiency) or hepatic disease (i.e., severe cirrhosis or end-stage liver disease)
- **Comorbidities**
  - Reduced medication dosages may apply to patients with renal disease (i.e., on dialysis or a diagnosis of chronic renal insufficiency) or hepatic disease (i.e., severe cirrhosis or end-stage liver disease)
- **Cultural and Psychological Sensitivity**
  - Communicate respectfully, considering language barriers and cultural barriers.
  - Provide reassurance and address patient/family anxiety to improve compliance and trust.
- **Infection Control**
  - Adhere to standard precautions (e.g., PPE, hand hygiene) to prevent cross-contamination.
  - Consider enhanced precautions for suspected infectious diseases (e.g., TB, MRSA)
- **Scene Management**
  - Co-ordinate with other emergency services (e.g., civil defense, police) to ensure efficient patient care and scene safety.
  - Consider extrication needs for trauma patients trapped in vehicles or confined spaces.
- **Critical Patients**
  - Ideally, one to two clinicians should be assigned to exclusively monitor and facilitate patient-focused care where possible.
  - Apart from lifesaving interventions needed to prevent deterioration, treatment should begin as soon as possible but must not slow down extrication or delay transport to definitive care.

- **Normal Vital Signs**[1]

Age	Pulse-Awake (beats/minute)	Pulse-Sleeping (beats/minute)	Respiratory Rate (breaths/minute)	Systolic BP (mmHg)
Preterm <1kg	120-160	-	30-60	39-59
Preterm 1-3kg	120-160	-	30-60	60-76
Newborn	100-205	85-160	30-60	67-84
Up to 1 year	100-190	90-160	30-60	72-104
1-2 years	100-190	90-160	24-40	86-106
2-3 years	98-140	60-120	24-40	86-106
3-4 years	80-140	60-100	24-40	89-112
4-5 years	80-140	60-100	22-34	89-112
5-6 years	75-140	58-90	22-34	89-112
6-10 years	75-140	58-90	18-30	97-115
10-12 years	75-118	58-90	18-30	102-120
12-13 years	60-100	58-90	15-20	110-131
13-15 years	60-100	50-90	15-20	110-131
15 years or older	60-100	50-90	15-20	110-131

  - Hypotension is considered a systolic blood pressure less than the lower limit of the chart
  - Hypertension is considered a systolic blood pressure higher than the higher limit on this chart. Hypertension may be an expected finding in many patients (e.g. anxiety, pain)
  - Tachycardia is considered a pulse above the upper limit on the chart
  - Bradycardia is considered a pulse below the lower limit on the chart
  - Tachypnea is considered a respiratory rate above the upper limit on the chart
  - Bradypnea is considered a respiratory rate below the lower limit on the chart.

- **Glascow Coma Scale (GCS)**[1,2-5]

Adult GCS And Pediatrics > 4 years	
Eye Opening	
Spontaneously	4
To Verbal Stimuli	3
To Pain Stimuli	2
No Response to Pain	1
Verbal Response	
Orientated and Converses	5
Confused and Converses	4
Inappropriate Words	3
Incomprehensible Words	2
No Response to Pain	1
Motor Response	
Obeys verbal Commands	6
Localizes to stimuli	5
Withdraws to stimuli	4
Abnormal flexion to pain (decorticate)	3
Abnormal extension to pain (decerebrate)	2
No Response to Pain	1

Pediatric GCS <4 years	
Eye Opening	
Spontaneously	4
To Verbal Stimuli	3
To Pain Stimuli	2
No Response to Pain	1
Verbal Response	
Appropriate words or social smile, fixes, follows cues	5
Cries but consolable; less than usual words	4
Persistently irritable	3
Moans to pain	2
No Response to Pain	1
Motor Response	
Obeys verbal Commands	6
Localizes to stimuli	5
Withdraws to stimuli	4
Abnormal flexion to pain (decorticate)	3
Abnormal extension to pain (decerebrate)	2
No Response to Pain	1

References

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Internal Policy References

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B. Dubai Corporation for Ambulance Services, 2024. DCAS Patient Assessment and Reassessment Policy. Document No. DCAS/CLIN/050. Dubai.